Vaccine talk

HSCT Recipient Vaccination Timeline (0–24 Months Post-Transplant)

The same schedule applies to autologous and allogeneic HSCT recipients.

Time from HSCT	0 months (Transplant)	3 months	4 months	6 months	12 months	18 months	24 months
Immune status considerations	Profound immunosuppression from conditioning therapy	Immune reconstitution begins but response to vaccines is often suboptimal	Immune recovery continues; some vaccines may be started based on risk	Most inactivated vaccines can be started if patient is clinically stable	Ongoing immune recovery	Continued immune recovery	Consider live vaccines if no GVHD, no immunosuppression and adequate immune reconstitution
Vaccines to administer ● Inactivated vaccines ● Live vaccines (see criteria below)	No routine vaccines during profound immunosuppression. Provide passive protection and infection prevention strategies.	• No routine vaccines yet. • Consider inactivated influenza vaccine in outbreak or high-transmission settings (not before 4 months routinely).	• Inactivated influenza vaccine (if in season) – may be considered. • If given now, a second dose should be considered.	• Pneumococcal vaccine (PCV20 or PCV15 series) • Hib (3-dose series) • DTaP/Tdap/Td* • Hepatitis B • Hepatitis A • IPV • Meningococcal ACWY • Meningococcal B** (from 2 months of age) • HPV (age 9–26 years, or 27–45 years based on shared decision-making) • Annual inactivated influenza vaccine	• Pneumococcal vaccine (complete series) • DTaP/Tdap/Td* (continue per age) • Hepatitis B (complete series) • Hepatitis A (complete series) • HPV (complete series) • Annual inactivated influenza vaccine • RZV*** (if eligible)	• DTaP/Tdap/Td* (complete) • Annual inactivated influenza vaccine • RZV*** (if not yet given and eligible)	• MMR (if eligible) • Varicella (if eligible) Continue: • Annual inactivated influenza vaccine • Catch-up for any missed doses
Live vaccines (criteria)	Do NOT *give live vaccines until ≥24 months post-HSCT AND:* ✓ No active GVHD ✓ No ongoing immunosuppressive therapy ✓ Adequate immune reconstitution						
	Live vaccines (MMR, varicella) may be given if all criteria met. Do not give live vaccines if GVHD or immunosuppression persists.						
Serologic testing (post-vaccination)	• **Hepatitis B:** check anti-HBs 4–6 weeks after completing series • **MMR:** check antibodies 4–6 weeks after 2nd dose						
	Serologic testing is NOT routinely recommended for: diphtheria, tetanus, pertussis, Hib, influenza, pneumococcal, polio, varicella (post-vaccination), COVID-19 vaccines.						

* Children <7 years: 3 doses DTaP; ≥7 years: 3 doses DTaP OR 1 dose Tdap + 2 doses DT OR 1 dose Tdap + 2 doses Td.

** Meningococcal B vaccine: routinely recommended from 2 months of age in at-risk individuals and adolescents as per national schedule.

*** RZV (Recombinant Zoster Vaccine): 3–12 months after autologous HSCT; 6–12 months after allogeneic HSCT.
Ideally complete ~2 months before stopping antiviral prophylaxis.

Note: These are general recommendations. Individual patient factors, local epidemiology and clinical judgement should guide decisions.